PathwayAI

Patient voices. County evidence. Practical next steps.

PathwayAI helps people organize the burden of illness for care and helps community teams explore county evidence and possible support actions. Patients and caregivers remain at the center; people review AI outputs, assumptions and decisions.

What works now

- Prepare for care: organize a story, review symptoms and timing, and download a personal visit-and-burden summary.
- Make hidden burdens visible: record payments, missed workdays, unpaid help and optional daily-life effects. Missing answers remain unknown. Caregiver observations are labeled separately.
- Prepare a support request: review a request for care access, transportation, costs, accessibility, food, housing or benefits help. Choose which personal details to include.
- Explore county evidence: compare Dutchess and Orange reported Lyme rates, public community context and labeled study-based cost illustrations.
- Plan a response: explore a need, county context, proposed action, possible participants and progress measures; download a planning brief.

How to read the information

Personal entries are reported by the person or caregiver, not verified clinical findings. Public evidence describes the source population and reporting period. Illustrations use invented records or explicit assumptions; they are not observed county spending, expected bills or proven savings. Workdays and caregiver hours remain separate from money unless an assumption is explicitly applied.

What still needs testing or development

The app does not diagnose, interpret uploaded lab reports, determine benefits eligibility, submit applications or guarantee services. Optional external story AI requires permission; the local organizer works without it. Public evidence AI does not receive private patient cards.

Real shared collection is off by default. Secure longitudinal records, consent-based aggregation, privacy-reviewed releases and partner evaluation remain future work. Fictional summaries demonstrate software behavior, not real participant findings or county prevalence. Source-verification gaps remain documented.

No faster diagnosis, improved health, causal savings or county adoption has been demonstrated.

Five-minute demonstration

Fictional story → reviewed personal card → support request → county action brief.

One planning question: should a county care-navigation team investigate a small ride-navigation pilot with willing residents? Patients and caregivers help choose the priority; partners would agree feasibility, responsibilities and follow-up measures before action.

Explore the public app: https://pathwayai.streamlit.app/

Code and evidence notes: https://github.com/gaiju/pathwayAI

Demonstration material prepared October 17, 2026. Local validation does not establish the live deployment's version or settings.